

BLOOD TESTS (GENERAL BLOOD WORK) — PATIENT GUIDE (UK)

(A4 Word-friendly plain text)

DOCUMENT PURPOSE

This guide explains why general blood tests are done, when they are usually requested, and what to do before and after your appointment.

1. WHAT IS A BLOOD TEST?

A blood test involves taking a small sample of blood (usually from a vein in your arm). The sample is sent to a laboratory to measure different parts of your blood and assess how your body is functioning.

2. WHY YOU MAY NEED GENERAL BLOOD TESTS

Blood tests can help to:

- check for anaemia (low red blood cells)
- look for infection or inflammation
- assess kidney and liver function
- check thyroid function
- monitor long-term conditions (e.g., kidney disease, thyroid disease)
- monitor medicines (some medicines require regular blood monitoring)
- investigate symptoms such as tiredness, weakness, dizziness, weight changes, or feeling unwell

3. COMMON GENERAL BLOOD TESTS (EXAMPLES)

- Full Blood Count (FBC): red and white blood cells and platelets
- Urea & Electrolytes (U&E): kidney function and salts in the blood
- Liver Function Tests (LFTs): liver health and bile flow markers
- Thyroid Function Tests (TFTs): underactive or overactive thyroid
- CRP or ESR: inflammation markers
- Ferritin / Iron studies: iron stores and iron deficiency
- Vitamin B12 / Folate / Vitamin D: vitamin levels linked to fatigue, bone and nerve health
- Clotting tests (e.g., INR): how quickly your blood clots (important for some medicines and before procedures)

4. WHEN TO TAKE WHICH TEST (TYPICAL GUIDANCE)

Your clinician decides which tests you need. Common situations include:

- Ongoing tiredness or dizziness: FBC, ferritin/iron studies, B12/folate, TFTs
- Suspected infection/inflammation: FBC, CRP/ESR
- Monitoring kidney disease, blood pressure medicines or diuretics: U&E
- Monitoring liver conditions or certain medicines: LFTs
- Before an operation: FBC, U&E, clotting tests (as requested)

5. BEFORE YOUR BLOOD TEST: PRECAUTIONS AND PREPARATION

A) CHECK IF YOU NEED TO FAST

Some blood tests require fasting (no food) for a set time, often 8–12 hours.

- Water is usually allowed during fasting unless you are told otherwise.
- If fasting is required, you will normally be asked to have only water and avoid tea/coffee with milk, sugary drinks, or snacks.

B) DRINK WATER (UNLESS TOLD NOT TO)

Being well-hydrated can make it easier to find a vein.

C) MEDICINES AND SUPPLEMENTS

- Keep taking your usual medicines unless your clinician tells you to stop.
- If you take blood thinners (e.g., warfarin, apixaban, rivaroxaban, dabigatran), tell the person taking your blood.
- If you take biotin (often in hair/skin/nails supplements), ask if you should stop it 24–48 hours before the test, as it can affect some results (especially thyroid tests).
- If you are unsure about any supplement (iron tablets, multivitamins, herbal products), mention them at your appointment.

D) ILLNESS ON THE DAY

If you have a fever or feel acutely unwell, ask your clinician whether the test should go ahead, as some results can change during illness.

E) WHAT TO WEAR

Wear clothing that allows easy access to the upper arm (short sleeves or loose sleeves).

6. DURING THE TEST: WHAT HAPPENS

- Your identity is checked (name and date of birth).
- A tourniquet (tight band) may be placed around your upper arm.

- The skin is cleaned.
- A small needle is inserted into a vein and blood is collected into one or more tubes.
- The needle is removed and you will be asked to press on cotton wool or a pad.

7. AFTER THE TEST: PRECAUTIONS AND AFTERCARE

- Keep pressure on the site for 1–2 minutes (longer if you bruise easily or take blood thinners).
- Keep the plaster on for at least 30 minutes.
- Avoid heavy lifting with that arm for a few hours.
- Mild bruising is common. A small lump can occur and usually settles.

GET URGENT ADVICE IF:

- bleeding does not stop after 10 minutes of firm pressure
- the area becomes increasingly swollen, hot, red, or very painful
- you feel faint and do not improve after resting and drinking water

8. RESULTS: HOW LONG THEY TAKE

Many routine results are available in 1–3 working days, but some specialist tests take longer. Ask how you will receive your results (text, app, call, or follow-up appointment).

END OF DOCUMENT

DIABETES TESTS (SCREENING, DIAGNOSIS AND MONITORING) — PATIENT GUIDE (UK)
(A4 Word-friendly plain text)

DOCUMENT PURPOSE

This guide explains the main diabetes tests, when they are used, and how to prepare.

1. WHY DIABETES TESTING IS DONE

Diabetes tests measure blood glucose (sugar) or show how well your body controls glucose. Testing may be done to:

- diagnose diabetes or pre-diabetes
- monitor diabetes control over time
- check risk if you have symptoms or relevant health conditions

2. WHO MAY NEED DIABETES TESTS

Testing may be advised if you have:

- excessive thirst, frequent urination, extreme tiredness
- unexplained weight loss, blurred vision
- recurrent infections or slow healing wounds
- history of gestational diabetes (diabetes in pregnancy)
- family history of diabetes, overweight, high blood pressure, high cholesterol, or certain ethnic risk factors

3. MAIN DIABETES-RELATED TESTS

A) HbA1c (Glycated Haemoglobin)

- What it shows: average blood glucose over the last 8–12 weeks
- Used for: diagnosis (in many cases) and long-term monitoring
- Fasting needed: No

B) Fasting Plasma Glucose (FPG)

- What it shows: blood glucose after fasting
- Used for: diagnosis and investigation of borderline glucose results
- Fasting needed: Yes (usually 8–12 hours; water is usually allowed)

C) Random Plasma Glucose

- What it shows: glucose level at any time of day
- Used for: assessment if symptoms are present
- Fasting needed: No

D) Oral Glucose Tolerance Test (OGTT)

- What it shows: how your body handles glucose after a glucose drink
- Used for: diagnosing diabetes/pre-diabetes; often used in pregnancy
- Fasting needed: Yes
- Typical process: fasting blood sample, glucose drink, blood sample again after a timed interval (often 2 hours)

E) Urine Albumin-to-Creatinine Ratio (ACR) (Kidney screening)

- What it shows: tiny amounts of protein in urine (early kidney damage)

- Used for: annual screening in many people with diabetes (or as advised)
- Sample: usually early morning urine sample if requested

4. WHEN TO TAKE WHICH TEST (TYPICAL GUIDANCE)

- New symptoms suggesting diabetes: HbA1c and/or fasting glucose; sometimes random glucose
- Borderline results or pregnancy assessment: OGTT
- Monitoring known diabetes: HbA1c every 3–6 months (as advised)
- Complication checks: urine ACR and kidney blood tests often yearly (or more often if abnormal)

5. BEFORE THE TEST: PRECAUTIONS AND PREPARATION

A) HbA1c

- No fasting needed. Eat and drink normally.

B) Fasting glucose and OGTT

- Fast for 8–12 hours (water is usually allowed).
- Avoid alcohol the night before.
- Avoid unusually heavy exercise the day before, as it can affect results.
- Continue medicines unless you are told otherwise. If you use insulin or diabetes tablets, ask your clinician for specific instructions for fasting days.
- If you are unwell (fever or infection), tell your clinician because illness can raise glucose.

6. DURING THE TEST: WHAT HAPPENS

- A blood sample is taken from your arm.
- For OGTT you will remain at the clinic/hospital, drink the glucose drink, and wait.
- During OGTT do not eat, smoke, or do vigorous activity unless staff say otherwise.

7. AFTER THE TEST: PRECAUTIONS AND AFTERCARE

- Bring a snack and drink for after fasting tests.
- If you feel faint, sit or lie down and inform staff.
- Follow any personalised medication advice you were given for the day of the test.

8. RESULTS AND FOLLOW-UP

Ask how and when you will get results, and what the next step is if results are abnormal (repeat testing, lifestyle plan, medication review, referral).

END OF DOCUMENT

CHOLESTEROL (LIPID PROFILE) TEST — PATIENT GUIDE (UK)
(A4 Word-friendly plain text)

DOCUMENT PURPOSE

This guide explains cholesterol testing, preparation, and what happens afterwards.

1. WHAT IS A CHOLESTEROL (LIPID) TEST?

A lipid profile measures fats in your blood, usually including:

- Total cholesterol
 - LDL cholesterol (often called “bad” cholesterol)
 - HDL cholesterol (often called “good” cholesterol)
 - Triglycerides
- Sometimes “non-HDL cholesterol” is also reported.

2. WHY A CHOLESTEROL TEST IS IMPORTANT

Abnormal cholesterol levels can increase the risk of:

- heart disease (including heart attacks)
- stroke
- peripheral arterial disease

3. WHEN YOU MAY NEED A CHOLESTEROL TEST

You may be offered a test if you:

- are having a routine health check
- have high blood pressure, diabetes, kidney disease, thyroid disease, or obesity
- smoke, or have a strong family history of early heart disease
- are starting or monitoring cholesterol-lowering treatment (e.g., statins)

4. WHEN TO TAKE WHICH LIPID TEST (TYPICAL GUIDANCE)

- Routine risk assessment: often a non-fasting lipid profile is acceptable

- Raised triglycerides or specialist assessment: fasting lipids may be requested
- After starting or changing statins: recheck often at 6–12 weeks, then periodically (as advised)

5. BEFORE THE TEST: PRECAUTIONS AND PREPARATION

- Check whether fasting is required. If fasting is needed: do not eat for 8–12 hours; water is usually allowed.
- Avoid heavy alcohol intake for 24–48 hours beforehand, especially if triglycerides are being assessed.
- Take your usual medicines unless told otherwise.
- In the days before the test, try to eat your normal diet (do not drastically change your diet just before testing).

6. DURING THE TEST

A small blood sample is taken from your arm. This usually takes only a few minutes.

7. AFTER THE TEST: PRECAUTIONS AND AFTERCARE

- Press on the site to reduce bruising.
- If you fasted, eat and drink normally afterwards.

8. WHAT HAPPENS IF RESULTS ARE HIGH?

Your clinician may recommend:

- dietary changes (reduce saturated fats; increase fibre; choose healthier fats)
- more physical activity and weight management
- reducing alcohol
- stopping smoking
- medication (e.g., statins), depending on your overall cardiovascular risk

END OF DOCUMENT

RADIOLOGY TESTS (X-RAY, ULTRASOUND, CT, MRI) — PATIENT GUIDE (UK)
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DOCUMENT PURPOSE

This guide explains common imaging tests, how to prepare, and what to do before and after.

IMPORTANT SAFETY NOTE

Always tell radiology staff if you might be pregnant, have allergies, kidney problems, diabetes, asthma, or implanted medical devices. Bring your appointment letter and follow its instructions, as preparation varies.

1. X-RAY

What it is: A quick scan using a small amount of radiation.

Common uses: fractures, chest and lung checks, joint problems.

Before:

- Usually no fasting needed.
- Remove jewellery/metal from the area being imaged.
- Tell staff if you might be pregnant.

During:

- You may need to stand, sit, or lie down.
- You may be asked to hold still or hold your breath briefly.

After:

- You can usually return to normal activities straight away.

2. ULTRASOUND

What it is: Uses sound waves (no radiation).

Common uses: abdomen, pelvis, pregnancy, kidneys, thyroid, soft tissue lumps.

Before (depends on the scan):

- Abdominal ultrasound: you may be asked not to eat for 4–6 hours before.
- Pelvic ultrasound: you may need a full bladder (drink water as instructed).
- Wear comfortable clothing and follow your appointment instructions.

During:

- Gel is applied to the skin.
- A handheld probe is moved over the area.
- Some pelvic scans use an internal probe; staff will explain and ask for consent.

After:

- Normal activities straight away.

3. CT SCAN (COMPUTED TOMOGRAPHY)

What it is: Detailed X-ray images; sometimes contrast dye is used.

Common uses: head injury, chest/abdominal problems, suspected clots, cancers.

Before:

- You may be asked to fast for a few hours if contrast is planned.
- Tell staff if you have kidney problems or have had a reaction to contrast dye.
- If you have diabetes and take metformin, ask for specific instructions before and after a contrast CT.
- Remove metal objects as directed.

During:

- You lie on a scanning table.
- If contrast is used, you may feel warmth or a metallic taste for a short time.

After:

- If you had contrast, drink fluids unless you have been told not to.
- Seek medical advice urgently if you develop breathing difficulty, swelling, rash, or feel very unwell after contrast (rare).

4. MRI SCAN (MAGNETIC RESONANCE IMAGING)

What it is: Uses a strong magnet (no radiation).

Common uses: brain, spine, joints, soft tissue.

Before:

- Tell staff if you have any implants or metal in your body, especially pacemakers/defibrillators, aneurysm clips, cochlear implants, or possible metal fragments in the eye.
- Remove all metal items (jewellery, watches, hair grips, piercings).
- If you are claustrophobic, contact the department in advance to discuss support options.
- Fasting is not always required, but follow your appointment letter, especially if contrast or sedation is planned.

During:

- The scanner is noisy; you will be given ear protection.
- You must lie still; the scan can take 15–60 minutes depending on the area.

After:

- Usually you can return to normal activity.
- If you had sedation, you will need someone to take you home and you should not drive for 24 hours (follow local instructions).

5. WHEN TO TAKE WHICH IMAGING TEST (GENERAL GUIDE)

- Suspected fracture or chest infection: X-ray
 - Gallbladder, abdominal organs, pregnancy/pelvic issues: ultrasound
 - Fast, detailed assessment of internal problems: CT
 - Detailed soft tissue assessment (brain/spine/joints) without radiation: MRI
- Your clinician chooses the test based on symptoms, urgency, and safety.

END OF DOCUMENT

NUTRITION TESTS (VITAMINS, MINERALS AND DEFICIENCIES) — PATIENT GUIDE (UK)
(A4 Word-friendly plain text)

DOCUMENT PURPOSE

This guide explains common nutrition-related blood and urine tests, when they are used, and how to prepare.

1. WHAT ARE NUTRITION-RELATED TESTS?

Nutrition tests help check whether your body has enough key vitamins, minerals and proteins, and whether you are absorbing nutrients properly. They are often used if you have tiredness, weight change, digestive symptoms, restricted diet, or long-term health conditions.

2. COMMON NUTRITION TESTS (EXAMPLES)

- Vitamin D (25-OH Vitamin D): bone and muscle health
- Vitamin B12 and Folate: nerve health and anaemia-related tiredness
- Iron studies (Ferritin, transferrin saturation): iron deficiency or overload
- Calcium, phosphate, magnesium: bone and muscle function

- Albumin / total protein: general health marker (can also reflect inflammation/illness)
- Coeliac screening (tTG IgA plus total IgA): checks for possible coeliac disease
- Sometimes added: thyroid tests, lipid profile, HbA1c (depending on symptoms and risk)

3. WHEN TO TAKE WHICH TEST (TYPICAL GUIDANCE)

- Tiredness, pallor, breathlessness, suspected anaemia: FBC, ferritin/iron studies, B12, folate
- Bone pain, muscle weakness, low sun exposure: vitamin D, calcium/phosphate
- Restricted diet (e.g., vegan), gut conditions, long-term metformin use: B12, folate, iron studies
- Digestive symptoms, bloating, diarrhoea, weight loss: coeliac screen and iron studies (plus others as advised)
- Metabolic risk or weight changes: lipids, HbA1c, liver tests (as advised)

4. BEFORE THE TEST: PRECAUTIONS AND PREPARATION

- Most nutrition blood tests do not require fasting, but follow any instructions on your appointment letter.
- Iron supplements: ask if you should stop iron tablets 24 hours before testing (some clinicians request this).
- Biotin supplements: may affect some blood test results; ask if you should stop 24–48 hours before.
- Coeliac blood test: you must be eating gluten regularly before the test (do not stop gluten beforehand unless told by your clinician), otherwise results may be unreliable.

5. AFTER THE TEST: PRECAUTIONS AND AFTERCARE

- Press on the site for 1–2 minutes to reduce bruising.
- Keep the plaster on for at least 30 minutes.
- Avoid heavy lifting with that arm for a few hours.

6. IF A DEFICIENCY IS FOUND

Your clinician may recommend:

- dietary changes and practical food sources

- supplements (dose and duration matched to your needs)
- further checks to find the cause (e.g., absorption problems or blood loss)
- repeat tests after treatment to confirm levels have improved

END OF DOCUMENT